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Question **1**

Not answered

Marked out of 1

What percentage of elderly patients with delirium will go on to develop dementia?

Select one:

- ☐ 40-50%
- ☐ 25-40%
- ☐ 10-25%
- ☐ 5%
- ☐ 1-2%

Your answer is incorrect.

Among the elderly 10-15% of patients have delirium on admission, and a further 10-40% develop delirium during their hospital stay. Liposwski (1983) reported no detectable cause for delirium in between 5 and 20% of cases.

The correct answer is: 5%

## Question 2

Not answered

Marked out of 1

The prevalence of dementia in the general population older than 65 years of age is approximately

Select one:

- ☐ 10%
- ☐ 5%
- ☐ 3-4%
- ☐ 15-20%
- ☐ 1-2%

Your answer is incorrect.

Explanation:

Dementia is a progressive syndrome affecting around 5% of those aged >65 years, rising to 20% in the over-80s. The most common cause of dementia is Alzheimer disease, accounting for around 60% of all cases.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 604.

The correct answer is: 5%

Question **3**

Not answered

Marked out of 1

In old age, what percentage of patients visit their General Practitioner a week before suicide?

Select one:

- ☐ 20%
- ☐ 50%
- ☐ 1%
- ☐ 5%
- ☐ 10%

Your answer is incorrect.

Approximately two-thirds of individuals who commit suicide have visited their physicians within one month before taking their lives; 40 to 50 percent have visited the week before.

The correct answer is: 50%



## Question 4

Not answered

Marked out of 1

The risk of vascular events in the elderly increases with use of atypical antipsychotics by a factor of approximately

Select one:

- ☐ 2
- ☐ 3
- ☐ 10
- ☐ 5
- ☐ 7

Your answer is incorrect.

Explanation:

Antipsychotics in the elderly have been associated with increased mortality (1-2% absolute risk), cerebrovascular events (rate ratio 1.73, range 1.6-1.87), metabolic side effects, and pneumonia. The increased risk of ischaemic stroke is thought to be similar for both atypical and typical antipsychotics.

Ref: Behrman S et al. Prescribing antipsychotics in older people: A mini-review. Maturitas. 2018 Oct 1;116:8-10.

The correct answer is: 2



Question **5**

Not answered

Marked out of 1

The microtubule-associated protein tau (MAPT) gene is located on chromosome

Select one:

- ☐ 19
- ☐ 14
- ☐ 11
- ☐ 21
- ☐ 17

Your answer is incorrect.

Explanation:

Tau protein is encoded by the MAPT (microtubule-associated protein tau) gene located on chromosome 17.

Ref: Barbier P et al. Role of Tau as a microtubule associated protein: structural and functional aspects. Frontiers in aging neuroscience. 2019;11:204.

The correct answer is: 17



## Question 6

Not answered

Marked out of 1

The gene for amyloid precursor protein (APP) is on the long arm of chromosome

Select one:

- ☐ 1
- ☐ 19
- ☐ 21
- ☐ 14
- ☐ All of the listed options

Your answer is incorrect.

Explanation:

The gene for amyloid precursor protein (APP) is on the long arm of chromosome 21. The process of differential splicing yields four forms of the APP. Whether the processing of abnormal APP is of primary causative significance in Alzheimer disease is unknown. However, many research groups are studying both the standard metabolic processing of APP and its processing in patients with dementia of the Alzheimer type to answer this question.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 240.

The correct answer is: 21

## Question 7

Not answered

Marked out of 1

The prevalence of paraphrenia in elderly above the age of 65 is

Select one:

- ☐ 1-2%
- ☐ 5-10%
- ☐ 0.02-0.05%
- ☐ 10-15%
- ☐ 15-20%

Your answer is incorrect.

People with late paraphrenia represent approximately 10% of the elderly population of psychiatric hospitals. The reported prevalence of the disorder among the elderly living in the community ranges from 0.1 to 4%, and its incidence has been estimated to be between 10-26 per 100 000 per year. (Almeida, O. P., Howard, R., Forstl, H., et al. (1992) Late paraphrenia: a review. International Journal of Geriatric Psychiatry, 7, 543-548.)

The correct answer is: 1-2%



## Question 8

Not answered

Marked out of 1

In elderly patients, what is the target serum lithium level for the treatment of bipolar disorder?

Select one:

- ☐ 0.8-1.0 mmol/L
- ☐ 1.0-1.2 mmol/L
- ☐ 0.2-0.4 mmol/L
- ☐ 0.4-0.7 mmol/L
- ☐ 1.5 mmol/L

Your answer is incorrect.

Explanation:

For adults with bipolar disorder, the standard lithium serum level should be 0.60-0.80mmol/L with the option to reduce it to 0.40-0.60mmol/L in case of good response but poor tolerance, or to increase it to 0.80-1.00mmol/L in case of insufficient response and good tolerance. For the elderly, a more conservative approach may be adopted: usually 0.40-0.60mmol/L, with the option to go to maximally 0.70 or 0.80mmol/L at 65-79 years and to maximally 0.70mmol/L over 80 years.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 297.

The correct answer is: 0.4-0.7 mmol/L



## Question 9

Not answered

Marked out of 1

The prevalence of depression in community samples over age 65 is approximately

Select one:

- ☐ 5 to 10%
- ☐ 20 to 25%
- ☐ 15 to 20%
- ☐ 1 to 2%
- ☐ 10 to 15%

Your answer is incorrect.

Community samples show a higher prevalence of depression especially in the elderly. In specific settings such as residential care homes, the prevalence may be still higher.

The correct answer is: 10 to 15%



## Question 10

Not answered

Marked out of 1

In the population aged over 65 the overall prevalence of clinically significant depression is estimated at

Select one:

- ☐ 2%
- ☐ 4-5%
- ☐ 20-25%
- ☐ 10-15%
- ☐ 1%

Your answer is incorrect.

In the population aged over 65 the prevalence of clinically significant depression is 10%. This figure is increased in hospital subpopulations and higher still in residential homes (Fountaulakis et al. 2003). Residential and nursing homes have a prevalence of 15-30%.

The correct answer is: 10-15%



Question **11**

Not answered

Marked out of 1

The proportion of patients with dementia for whom the condition is attributable to vascular causes is

Select one:

- ☐ 1-2%
- ☐ 5-10%
- ☐ 20-25%
- ☐ 50-60%
- ☐ 40-50%

Your answer is incorrect.

Explanation:

The most common cause of dementia is Alzheimer disease, accounting for around 60% of all cases. Vascular dementia and dementia with Lewy bodies are responsible for most other cases. Vascular dementia is the second most common cause, accounting for 20% of dementia.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 604.

Samuel D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 164.

The correct answer is: 20-25%

## Question 12

Not answered

Marked out of 1

The estimated proportion of acute confusional states in hospitals that are considered preventable is

Select one:

- ☐ 10-15%
- ☐ 30-40%
- ☐ 1-5%
- ☐ 5-10%
- ☐ 20-25%

Your answer is incorrect.

Explanation:

Almost 40% of all cases of delirium in hospitals may be preventable.

Ref: Johansson YA et al. Delirium in older hospitalized patients—signs and actions: a retrospective patient record review. BMC geriatrics. 2018 Dec 1;18(1):43.

The correct answer is: 30-40%



## Question 13

Not answered

Marked out of 1

The 10-year probability of conversion from Mild Cognitive Impairment to dementia is

Select one:

- ☐ 30-40%
- ☐ 0.03-0.06%
- ☐ 1-2%
- ☐ 45-55%
- ☐ 60-70%

Your answer is incorrect.

Mitchell and Shiri-Feshki (2009) conducted a meta-analysis of 41 high-quality studies and noted that the yearly conversion rate from MCI to dementia is around 5-10%; even after 10 years, only 40% (not the majority) of MCI patients will progress to dementia. Other meta-analyses of long-term (5-10 years) studies present still lower yearly conversion rates, varying between 3.3 – 4.2%, with cumulative conversion rates of ~31% over ten years (<http://www.ncbi.nlm.nih.gov/pubmed/19236314>).

The correct answer is: 30-40%

Question **14**

Not answered

Marked out of 1

A 75-year-old man has amnesic mild cognitive impairment. What is the likelihood of him developing Alzheimer dementia within a year's time?

Select one:

- ☐ 10%
- ☐ 20%
- ☐ 30%
- ☐ 40%
- ☐ 50%

Your answer is incorrect.

Explanation:

Around 10% of elderly individuals with MCI will progress to dementia in one year.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 159.

Eshkoor SA et al. Mild cognitive impairment and its management in older people. Clinical interventions in aging. 2015;10:687.

The correct answer is: 10%

## Question 15

Not answered

Marked out of 1

A 79-year-old woman is admitted to the psychiatric ward while experiencing paranoid psychosis. She is started on chlorpromazine. How much higher is her risk for developing tardive dyskinesia compared to a 33-year-old woman treated for the same condition with the same dose?

Select one:

- ☐ 5-6 times
- ☐ 10 times
- ☐ 3-4 times
- ☐ 2 times
- ☐ 15 times

Your answer is incorrect.

Explanation:

Tardive dyskinesia is a common iatrogenic neurological and neurobehavioural syndrome associated with the use of antidopaminergic medication, especially antipsychotics. Age is a significant risk factor for the development of tardive dyskinesia. The older the patient, the greater the risk. There is up to a 6-fold greater incidence in those exposed to antipsychotics in mid- to late-life compared to younger adults.

Ref: Owens DC. Tardive dyskinesia update: the syndrome. BJPsych Advances. 2019 Jan;25(1):57-69.

The correct answer is: 5-6 times

Question **16**

Not answered

Marked out of 1

What is the average life expectancy of patients diagnosed with Lewy body dementia?

Select one:

- ☐ 15 years
- ☐ 6 years
- ☐ 1 year
- ☐ 2 years
- ☐ 3 years

Your answer is incorrect.

Explanation:

Dementia with Lewy bodies is the third most common type of dementia (5-10%). It is characterised by progressive dementia with parkinsonism, as well as fluctuations in levels of attention and severity of cognitive impairment. Life expectancy/mean survival time after diagnosis is approximately 6 years, though it is variable (generally 5.5-7.7 years from disease onset, 1.9-6.3 years from diagnosis).

Ref: Mueller C, Ballard C, Corbett A, Aarsland D. The prognosis of dementia with Lewy bodies. The Lancet Neurology. 2017 May 1;16(5):390-8.

The correct answer is: 6 years



## Question 17

Not answered

Marked out of 1

Very-late-onset schizophrenia-like psychosis is characterised by onset after

Select one:

- ☐ 60 years of age
- ☐ 50 years of age
- ☐ 40 years of age
- ☐ 80 years of age
- ☐ 70 years of age

Your answer is incorrect.

Explanation:

Schizophrenia is currently subdivided into early-onset (EOS, onset before age 40), late-onset (LOS, onset age 40-60), and very-late-onset schizophrenia-like psychosis (VLOS, onset after 60).

Ref: Tampi RR et al. Psychotic disorders in late life: a narrative review. Therapeutic advances in psychopharmacology. 2019 Oct;9:1-13.

The correct answer is: 60 years of age

Question **18**

Not answered

Marked out of 1

Among elderly patients admitted to the intensive care unit, the prevalence of delirium is around

Select one:

- ☐ 1-2%
- ☐ 50-60%
- ☐ 80-85%
- ☐ 15-20%
- ☐ 4-6%

Your answer is incorrect.

Explanation:

The prevalence of delirium is around 20% among general medical admissions, over 50% in medical admissions of older people, and over 80% in the intensive care unit and palliative care.

Ref: FitzGerald JM, Price A. Delirium in the acute hospital setting: the role of psychiatry. BJPsych Advances. 2020 Jul 30:1-11.

The correct answer is: 80-85%



## Question 19

Not answered

Marked out of 1

Which of the following is true with respect to late onset schizophrenia?

Select one:

- ☐ 20% have thought insertion
- ☐ 30% have thought broadcasting
- ☐ 30% have negative symptoms
- ☐ 10% have auditory hallucinations
- ☐ 90% have persecutory delusions

Your answer is incorrect.

Persecutory delusions are the most common symptoms of late paraphrenia; they are found in around 90% of patients (Almeida et al., 1995a). Auditory hallucinations occur in approximately 75% of cases.

The correct answer is: 90% have persecutory delusions



Question **20**

Not answered

Marked out of 1

Characteristic features of very-late-onset schizophrenia-like psychosis include all except a

Select one:

- ☐ Lower likelihood of family history of schizophrenia
- ☐ Lower likelihood of affective blunting
- ☐ Greater risk of developing tardive dyskinesia
- ☐ Greater likelihood of formal thought disorder
- ☐ Greater likelihood of visual hallucinations

Your answer is incorrect.

Explanation:

Compared with early- or late-onset schizophrenia, very-late-onset schizophrenia-like psychosis is characterised by greater likelihood of sensory impairment, social isolation, visual hallucinations, lesser likelihood of formal thought disorder and affective blunting, lesser likelihood of family history of schizophrenia, and greater risk of developing tardive dyskinesia.

| <b>SPMM COURSE</b>  | <b>Early-onset schizophrenia (EOS)</b> | <b>Late-onset schizophrenia (LOS)</b> | <b>Very-late-onset schizophrenia-like psychosis (VLOS)</b> |
|------------------------------------------------------------------------------------------------------|----------------------------------------|---------------------------------------|------------------------------------------------------------|
| <b>Age of onset</b>                                                                                  | < 40 years                             | 40-60 years                           | > 60 years                                                 |
| <b>Female preponderance</b>                                                                          | No                                     | Yes                                   | Definitely                                                 |
| <b>Negative symptoms</b>                                                                             | Definitely                             | May be present                        | Less likely                                                |
| <b>Learning and retention</b>                                                                        | Ok                                     | Ok                                    | Impaired                                                   |
| <b>Progressive cognitive deterioration</b>                                                           | Yes                                    | Yes                                   | Yes (very high)                                            |
| <b>Brain structural abnormalities</b>                                                                | No                                     | No                                    | Yes                                                        |
| <b>Family history</b>                                                                                | Present                                | Present                               | Weak association                                           |
| <b>Early childhood maladjustments</b>                                                                | Present                                | Present                               | Absent                                                     |
| <b>Antipsychotic dosing</b>                                                                          | Higher                                 | Lower                                 | Lower                                                      |
| <b>Risk of tardive dyskinesia</b>                                                                    | Present                                | Present                               | Very high                                                  |

Adapted from Tampi et al. 2019

Ref: Tampi RR et al. Psychotic disorders in late life: a narrative review. Therapeutic advances in psychopharmacology. 2019 Oct;9:1-13.  
 Gautam S et al. Clinical practice guideline for management of psychoses in elderly. Indian journal of psychiatry. 2018 Feb;60(Suppl 3):S363.  
 The correct answer is: Greater likelihood of formal thought disorder

Question **21**

Not answered

Marked out of 1

According to the UK National Psychiatric Morbidity Survey, the factor NOT associated with the risk of a neurotic disorder is

Select one:

- ☐ Age 55-64
- ☐ Female sex
- ☐ Loneliness
- ☐ Divorced
- ☐ Unemployment

Your answer is incorrect.

Age 55-64 is not associated with the risk of a neurotic disorder.

The correct answer is: Age 55-64



Question **22**

Not answered

Marked out of 1

You are seeing a 76-year-old gentleman in the memory clinic. On routine blood tests, his calcium levels are noted to be high. Low levels of which of the following can artificially elevate calcium in the blood?

Select one:

- ☐ Urea
- ☐ Phosphate
- ☐ Creatinine
- ☐ Sodium
- ☐ Albumin

Your answer is incorrect.

The amount of total calcium varies with the level of serum albumin, a protein to which calcium is bound. So when albumin levels are low, free calcium levels will be high in the plasma as they are not bound to proteins. Therefore low levels of albumin can artificially elevate calcium levels in the blood. Low albumin levels (hypoalbuminaemia) may be caused by various conditions such as liver disease, nephrotic syndrome, malabsorption, malnutrition (common in older people with dementia), burns, late pregnancy, artefact, genetic variations and malignancy.

The correct answer is: Albumin



Question **23**

Not answered

Marked out of 1

Which of the following is consistently found in normal ageing brains?

Select one:

- ☐ Lewy bodies in the substantia nigra and locus cereleus
- ☐ Tau proteins in some nerve cells
- ☐ Lipofuscin in the cytoplasm of nerve cells
- ☐ Hirano bodies in new hippocampal pyramidal cells
- ☐ All of the listed options

Your answer is incorrect.

Explanation:

Population-based autopsy studies of the brains of aged people **without** a neurological disease consistently report the presence of amyloid plaques, neurofibrillary tangles, Lewy bodies, inclusion of TAR DNA-binding protein 43 (TDP-43), synaptic dystrophy, the loss of neurons, and the loss of brain volume in most of the brains. As well as classic protein deposits, other subcellular structures that accumulate in ageing brains (inside glial cells or neuronal cells) include stress granules, lipofuscin, Marinesco bodies, and Hirano bodies.

Ref: Wyss-Coray T. Ageing, neurodegeneration and brain rejuvenation. Nature. 2016 Nov;539(7628):180-6.

The correct answer is: All of the listed options



Question **24**

Not answered

Marked out of 1

Factors contributing to an increased risk of psychosis in the elderly include

Select one:

- ☐ Age-related deterioration of frontal and temporal cortices
- ☐ Neurochemical changes associated with ageing
- ☐ Age-related pharmacokinetic and pharmacodynamic changes
- ☐ Cognitive decline
- ☐ All of the listed options

Your answer is incorrect.

Explanation:

Factors that contribute to an increased risk of developing psychotic symptoms in old age include age-related deterioration of frontal and temporal cortices, age-related pharmacokinetic and pharmacodynamic changes, neurochemical changes associated with ageing, social isolation, sensory deficits, cognitive decline, and polypharmacy.

Ref: Gautam S et al. Clinical practice guideline for management of psychoses in elderly. Indian journal of psychiatry. 2018 Feb;60(Suppl 3):S363.

The correct answer is: All of the listed options

## Question 25

Not answered

Marked out of 1

Which of the following statements about delirium in palliative care is incorrect?

Select one:

- ☐ It is irreversible.
- ☐ It always responds well to neuroleptics.
- ☐ It is multifactorial in origin.
- ☐ It occurs in over 50% of patients at some stage in their illness.
- ☐ It may present with sedation and hypoactivity.

Your answer is incorrect.

Explanation:

Delirium is a common complication encountered during palliative care, occurring in over 50% of cases. Patients may become agitated or aggressive or alternatively, may present with hypoactive delirium. Antipsychotics may reduce the severity and duration of delirium or may further cloud the patient's mental state.

Ref: Aziz VM, Saeed R. Palliative care for older people: the psychiatrist's role. BJPsych Advances. 2019 Jan;25(1):37-46.

The correct answer is: It always responds well to neuroleptics.

Question **26**

Not answered

Marked out of 1

A 70-year-old man reports hearing 'burglar noises' in his attic every night. He believes that his neighbours are spying on him and are planning a 'coup' to evacuate him from his bungalow. He seems saddened by this problem, but has no somatic signs. The most likely cause of such a late-onset psychosis is

Select one:

- ☐ Alzheimer dementia
- ☐ Paranoid personality disorder
- ☐ Very-late-onset schizophrenia-like psychosis
- ☐ Late-onset schizophrenia
- ☐ Substance misuse

Your answer is incorrect.

Explanation:

The overall pooled prevalence of psychotic symptoms among older individuals with major neurocognitive disorders (e.g. due to Alzheimer disease) is as high as 40%.

Ref: Tampi RR et al. Psychotic disorders in late life: a narrative review. Therapeutic advances in psychopharmacology. 2019 Oct;9:1-13.

The correct answer is: Alzheimer dementia

Question **27**

Not answered

Marked out of 1

Which of the following is a risk factor for psychiatric disorders in the elderly?

Select one:

- ☐ high neuroticism
- ☐ Ambivalent religious beliefs
- ☐ inadequately controlled physical symptoms, especially pain
- ☐ distressing physical symptomatology
- ☐ feeling a burden on the family

Your answer is incorrect.

Uncertain, ambivalent religious beliefs are indicated as risk factors for psychiatric morbidity in the elderly (CR104: RCPsych)

The correct answer is: Ambivalent religious beliefs



Question **28**

Not answered

Marked out of 1

Which of the following drugs interacts with lithium and reduces its serum levels?

Select one:

- ☐ Fluoxetine
- ☐ Angiotensin II antagonists
- ☐ ACE inhibitors
- ☐ Indomethacin
- ☐ Aminophylline

Your answer is incorrect.

Explanation:

Aminophylline decreases proximal tubule sodium reabsorption in the kidney, increasing lithium excretion and decreasing serum lithium levels.

Ref: Timmer RT, Sands JM. Lithium intoxication. Journal of the American Society of Nephrology. 1999 Mar 1;10(3):666-74.

The correct answer is: Aminophylline

## Question 29

Not answered

Marked out of 1

Which of the following personality traits is associated with suicide in people aged over 60 years?

Select one:

- ☐ Histrionic
- ☐ Paranoid
- ☐ Anxious
- ☐ Schizoid
- ☐ Dependent

Your answer is incorrect.

A case-control study using retrospective information of personality traits showed that anankastic and anxious personality disorders are predictors of suicide in older people. (Harwood et al. 2001)

The correct answer is: Anxious



Question **30**

Not answered

Marked out of 1

Of those listed, which personality disorder is most associated with suicide in older adults?

Select one:

- ☐ Paranoid
- ☐ Schizoid
- ☐ Dependent
- ☐ Obsessive-compulsive
- ☐ Borderline

Your answer is incorrect.

Explanation:

Obsessive-compulsive and avoidant personality disorders have been implicated in death by suicide in older adults, although this association may be confounded by depression. An inability to adapt to the changes occurring in late life may help to explain the association between suicide in old age and higher conscientiousness as well as obsessive-compulsive and avoidant personality disorders. Cluster B personality disorders and their defining constructs play a smaller role in late-life suicide compared to suicide in younger adults.

Ref: Szücs A et al. Personality and suicidal behavior in old age: a systematic literature review. Frontiers in psychiatry. 2018 May 7;9:128.

The correct answer is: Obsessive-compulsive

Question **31**

Not answered

Marked out of 1

Which of the following features is seen more in the cortical type of dementia than the subcortical type?

Select one:

- ☐ Apathy
- ☐ Slowed motor speed and control
- ☐ Aphasia in early stages
- ☐ Preserved calculation
- ☐ Adventitious movements

Your answer is incorrect.

Explanation:





| Characteristic                 | Subcortical dementia           | Cortical dementia                            |
|--------------------------------|--------------------------------|----------------------------------------------|
| Language                       | No aphasia (anomia, if severe) | Aphasia early                                |
| Memory                         | Impaired recall>recognition    | Recall and recognition impaired              |
| Attention and immediate recall | Impaired                       | Impaired                                     |
| Visuospatial skills            | Impaired                       | Impaired                                     |
| Calculation                    | Preserved until late           | Involved early                               |
| Executive function             | Disproportionately affected    | Impairment consistent with other involvement |
| Speed of cognitive processing  | Slowed early                   | Normal until late in disease                 |
| Personality                    | Apathetic, inert               | Unconcerned                                  |
| Mood                           | Depressed                      | Euthymic                                     |
| Speech                         | Dysarthric                     | Articulate until late                        |
| Posture                        | Bowed or extended              | Upright                                      |
| Coordination                   | Impaired                       | Normal until late                            |
| Motor speed and control        | Slowed                         | Normal                                       |
| Adventitious movements         | Chorea, tremor, tics, dystonia | Absent (Alzheimer dementia –some myoclonus)  |
| Abstraction                    | Impaired                       | Impaired                                     |

Adapted from Kaplan and Sadock, 12<sup>th</sup> Ed. 2022. p. 244.



Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 244.

The correct answer is: Aphasia in early stages

Question **32**

Not answered

Marked out of 1

Which one of the following is a proven protective factor in late onset familial dementia of Alzheimer's type?

Select one:

- ☐ Smoking
- ☐ Apolipoprotein e4 allele
- ☐ Premorbid intelligence and education
- ☐ NSAIDs
- ☐ Apolipoprotein e2 allele

Your answer is incorrect.

Being a carrier of apolipoprotein e2 allele can be a protective factor against the development of Alzheimer's dementia.

The correct answer is: Apolipoprotein e2 allele



Question **33**

Not answered

Marked out of 1

What is the mode of inheritance of Pick's disease?

Select one:

- ☐ Point mutation
- ☐ X-linked recessive
- ☐ X-linked dominant
- ☐ Autosomal dominant
- ☐ Autosomal recessive

Your answer is incorrect.

A proportion of Pick's disease cases is familial though the majority are sporadic with no other family members being affected. In the familial cases, it is known as an 'autosomal dominant' disease and tends to affect people by about 40 years of age. There is strong family history in 50% of cases. Genetics: Associations with 17q21-22 (familial inheritance) and mutations in the tau gene

The correct answer is: Autosomal dominant



Question **34**

Not answered

Marked out of 1

Which of the following is widely used to summarize information on activities of daily living (ADLs) in the elderly?

Select one:

- ☐ Clifton Assessment Procedure for the Elderly
- ☐ Cambridge Diagnostic Examination
- ☐ Abbreviated Geriatric Depression Scale
- ☐ Barthel Index
- ☐ Geriatric Mental State Schedule

Your answer is incorrect.

Explanation:

The Barthel Index is a widely used measure of ADL function (self-maintenance skills such as dressing, bathing, and grooming) that has the advantages of simplicity, communicability, and ease of scoring. It has been validated and is used globally.

Ref: Yi Y et al. Is Barthel Index suitable for assessing activities of daily living in patients with dementia?. Frontiers in Psychiatry. 2020;11:282.

The correct answer is: Barthel Index



## Question 35

Not answered

Marked out of 1

Factors associated with very-late-onset schizophrenia-like psychosis include all of the following except

Select one:

- ☐ Female sex
- ☐ Visual impairment
- ☐ Hearing loss
- ☐ Social isolation
- ☐ Below average educational achievement

Your answer is incorrect.

Explanation:

Compared with early- or late-onset schizophrenia, very-late-onset schizophrenia-like psychosis is characterised by greater likelihood of sensory impairment, social isolation, visual hallucinations, lesser likelihood of formal thought disorder and affective blunting, lesser likelihood of family history of schizophrenia, and greater risk of developing tardive dyskinesia. There is a preponderance of women, with an accelerating incidence rate after age 80.

Ref: Tampi RR et al. Psychotic disorders in late life: a narrative review. Therapeutic advances in psychopharmacology. 2019 Oct;9:1-13.

Gautam S et al. Clinical practice guideline for management of psychoses in elderly. Indian journal of psychiatry. 2018 Feb;60(Suppl 3):S363.

The correct answer is: Below average educational achievement

Question **36**

Not answered

Marked out of 1

An elderly patient initially presented with anxiety and insomnia, but became sexually disinhibited after taking a newly prescribed medication. What is the most likely medication?

Select one:

- ☐ Sodium valproate
- ☐ A benzodiazepine
- ☐ Olanzapine
- ☐ Mirtazapine
- ☐ Carbamazepine

Your answer is incorrect.

Explanation:

The elderly are at increased risk of experiencing disinhibitory reactions secondary to benzodiazepines. Reactions may include acute excitement, hyperactivity, increased anxiety, vivid dreams, sexual disinhibition, aggression, hostility, and rage.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 448.

The correct answer is: A benzodiazepine

Question **37**

Not answered

Marked out of 1

Two years ago, a patient had a large haemorrhagic stroke that resolved with treatment. He now has vascular dementia. The most likely neurological finding in this patient is

Select one:

- ☐ Circumduction gait
- ☐ Bilateral lower limb muscle wasting
- ☐ Bilateral Babinski signs
- ☐ Unilateral brisk deep tendon reflexes
- ☐ Pupillary defects

Your answer is incorrect.

Explanation:

Focal neurological findings, such as asymmetrical hyperreflexia or weakness, are seen more often in vascular than in degenerative dementia.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 236.

The correct answer is: Unilateral brisk deep tendon reflexes

Question **38**

Not answered

Marked out of 1

The most common cause of death in patients with dementia of Alzheimer type is

Select one:

- ☐ Seizure
- ☐ Urinary tract infection
- ☐ Pneumonia
- ☐ Meningitis
- ☐ Cellulitis

Your answer is incorrect.

Explanation:

The most common cause of death in Alzheimer dementia, particularly in advanced disease, is pneumonia.

Ref: O'Brien et al. Clinical practice with anti-dementia drugs: a revised (third) consensus statement from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2017 Feb;31(2):147-68.

The correct answer is: Pneumonia





## Question 39

Not answered

Marked out of 1

Which of the following is used for the differential diagnosis of cognitive dysfunction in older people?

Select one:

- ☐ Mini mental state examination
- ☐ Alzheimer disease assessment scale
- ☐ Cambridge examination for mental disorders
- ☐ Short portable mental status questionnaire
- ☐ Mattis dementia rating scale

Your answer is incorrect.

Explanation:

The Cambridge Examination for Mental Disorders of the Elderly (CAMDEX-R) is a well-organised and generally comprehensive instrument for the differential diagnosis of dementia. CAMDEX was designed specifically for use in elderly people with the diagnosis of dementia and was later revised as CAMDEX-R. It includes a structured interview with the patient; a relative or other informant interview; a brief physical examination; and a neuropsychological test battery, the Cambridge Cognitive Assessment - Revised (CAMCOG-R).

Ref: Hodges JR. Cognitive assessment for clinicians, 3<sup>rd</sup> Edition. 2018. pp. 158-161.

The correct answer is: Cambridge examination for mental disorders

Question **40**

Not answered

Marked out of 1

Which scale is useful to obtain a differential diagnosis of dementia among the elderly?

Select one:

- ☐ CARE
- ☐ CAPE
- ☐ MMSE
- ☐ CAMDEX
- ☐ GMS

Your answer is incorrect.

The CAMDEX incorporates MMSE, Blessed dementia rating scale and Haschinski Ischemia score, in a comprehensive inquiry from the subject and informant. BDRS is a measure of activities of daily living. The Hachinski Ischemia Score helps to differentiate between vascular and Alzheimer's dementia. A notable feature of CAMDEX is CAMCOG, which is the cognitive subscale. Trained non-doctors mainly use CAMDEX in research. (Seminars in old age psychiatry-14)

The correct answer is: CAMDEX



Question **41**

Not answered

Marked out of 1

Mr. Z is a 75-year-old man seen at the psychogeriatric clinic for cognitive and language difficulties. His speech is characterised by slurring of consonants accompanied by a staccato pattern. Which of the following is most likely to account for this type of presentation?

Select one:

- ☐ Supranuclear palsy
- ☐ Motor neuron disease
- ☐ Cerebellar degeneration
- ☐ Myasthenia gravis
- ☐ Parkinson disease

Your answer is incorrect.

Explanation:

Cerebellar dysarthria has a typical scanning (explosive with staccato) feature, voice has a nasal character, and speech is slurred.

Ref: Manto M. Cerebellar motor syndrome from children to the elderly. In Handbook of clinical neurology 2018 Jan 1 (Vol. 154, pp. 151-166). Elsevier.

The correct answer is: Cerebellar degeneration

## Question 42

Not answered

Marked out of 1

Which of the following features is not required for a probable diagnosis of Alzheimer's disease according to the NINCDS/ADRDA Criteria? (National institute of neurological and communicative disorders and stroke/Alzheimer's disease and Related disorders association)

Select one:

- ☐ An absence of other disorders causing dementia
- ☐ Progressive decline in memory
- ☐ No alteration in consciousness
- ☐ Abnormal cognitive function on formal testing
- ☐ Cerebral atrophy on a CT head scan

Your answer is incorrect.

According to NINCDS-ADRDA criteria proposed in 1984, for a probable Alzheimer's disease, dementia should be established by clinical and neuropsychological examination. Cognitive impairments also have to be progressive and be present in 2 or more areas of cognition. The onset of the deficits has been between the ages of 40 and 90 years and finally there must be an absence of other diseases capable of producing a dementia syndrome. For a definite diagnosis of Alzheimer's disease, the patient should meet the criteria for probable Alzheimer's disease with histopathologic evidence of Alzheimer's disease (autopsy).

The correct answer is: Cerebral atrophy on a CT head scan

Question **43**

Not answered

Marked out of 1

Which of the following scales includes an assessment of the activities of daily living?

Select one:

- ☐ CATEGO
- ☐ Neuropsychiatric Inventory
- ☐ Clinical Dementia Rating scale
- ☐ SCAN
- ☐ AGE CAT

Your answer is incorrect.

Explanation:

Clinical Dementia Rating scale (CDR):

- allows for more reliable staging of dementia than MMSE
- based on caregiver accounts of problems in daily functional and cognitive tasks
- takes only a few minutes for clinicians already familiar with individual cases
- classifies people with dementia into questionable, mild, moderate, and severe

Ref: Sheehan B. Assessment scales in dementia. Therapeutic advances in neurological disorders. 2012 Nov;5(6):349-58.

The correct answer is: Clinical Dementia Rating scale

Question **44**

Not answered

Marked out of 1

All of the following features correlate with the severity of Alzheimer dementia except

Select one:

- ☐ Declining Mini Mental State Examination scores
- ☐ Clinically evident depression
- ☐ Declining activities of daily living scores
- ☐ Declining language function
- ☐ Neurological features such as seizures and myoclonus

Your answer is incorrect.

Explanation:

The stage of AD dementia may impact the occurrence of depression. Depression has been shown to become more frequent as AD progresses from mild to moderate dementia, and to be less common in severe dementia.

Ref: Burke AD et al. Diagnosing and treating depression in patients with Alzheimer's disease. Neurology and therapy. 2019 Aug 21:1-26.

The correct answer is: Clinically evident depression

## Question 45

Not answered

Marked out of 1

A 76-year-old man presents with a 3-year history of memory and language difficulties. Which of the following neurological findings would make a diagnosis of previous cerebrovascular disease more likely?

Select one:

- ☐ Diminished power of the lower limbs
- ☐ Clonus
- ☐ Absent bilateral knee jerks
- ☐ Muscle wasting of lower left leg
- ☐ Facial weakness with inability to raise the eyebrows

Your answer is incorrect.

Explanation:

Physical signs of vascular disease may include focal neurological signs, rigidity, akinesia, brisk reflexes/clonus, and pseudobulbar palsy.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 165.

Van Der Flier WM et al. Vascular cognitive impairment. Nature Reviews Disease Primers. 2018 Feb 15;4(1):1-6.

The correct answer is: Clonus

Question **46**

Not answered

Marked out of 1

Which of the following is false with respect to personality disorders in old age?

Select one:

- ☐ Cluster B disorders are the most common
- ☐ Rates of personality disorders is highest among those with depression
- ☐ Personality disorders can be diagnosed after the age of 65
- ☐ Personality disorders contribute to treatment resistance in depression
- ☐ Diogenes syndrome is considered as an end-stage of personality disorder

Your answer is incorrect.

The diagnostic criteria for personality disorders are somewhat age-biased as they require evidence of dysfunction from younger ages - this information may not be available when diagnosing personality disorders in elderly for the first time. Nevertheless, there is no restriction for making such diagnoses in the elderly. No specific cluster differences are noted in the prevalence rates of personality disorders in the elderly though a lower prevalence of cluster B is suggested.

The correct answer is: Cluster B disorders are the most common





Question **47**

Not answered

Marked out of 1

Which of the following is not a subcortical type of dementia?

Select one:

- ☐ Huntington disease
- ☐ Binswanger disease
- ☐ Parkinson disease
- ☐ Creutzfeldt-Jakob disease
- ☐ HIV-associated dementia

Your answer is incorrect.

Explanation:

## Classification of dementias

| Cortical                                                                                                                         | Subcortical                                                                                                                                                                                                                                                                                                                                           | Cortico-subcortical                                                                                                                                                                      |
|----------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <ul style="list-style-type: none"><li>• Alzheimer disease</li><li>• Frontotemporal dementia (including "Pick disease")</li></ul> | <ul style="list-style-type: none"><li>• Vascular dementia (including Biswanger disease)</li><li>• Progressive supranuclear palsy</li><li>• Multiple system atrophy</li><li>• Huntington disease</li><li>• Parkinson disease</li><li>• Wilson disease</li><li>• Multiple sclerosis</li><li>• Hydrocephalus</li><li>• HIV-associated dementia</li></ul> | <ul style="list-style-type: none"><li>• Dementia with Lewy bodies</li><li>• Creutzfeldt-Jacob disease (multi-focal, rapid, and aggressive)</li><li>• Corticobasal degeneration</li></ul> |

Adapted from Shaik and Varma 2012



Ref: Shaik SS, Varma AR. Differentiating the dementias: a neurological approach. Progress in Neurology and Psychiatry. 2012 Jan;16(1):11-8.  
The correct answer is: Creutzfeldt-Jakob disease

Question **48**

Not answered

Marked out of 1

Periodic complexes on EEG is a feature seen in

Select one:

- ☐ Creutzfeldt-Jakob disease
- ☐ Delirium
- ☐ Frontotemporal dementia
- ☐ Binswanger encephalopathy
- ☐ Huntington disease

Your answer is incorrect.

Explanation:

The EEG in Creutzfeldt-Jakob disease shows 'periodic complexes' - periodic bi- or triphasic discharges against slight low voltage background.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 168.

The correct answer is: Creutzfeldt-Jakob disease



## Question 49

Not answered

Marked out of 1

Abrupt onset and fluctuating course are typical and highly suggestive of

Select one:

- ☐ Depression
- ☐ Alzheimer dementia
- ☐ CJD
- ☐ Pseudo-dementia
- ☐ Delirium

Your answer is incorrect.

Explanation:

Delirium is characterised by disturbed attention (i.e., reduced ability to direct, focus, sustain, and shift attention) and awareness (i.e., reduced orientation to the environment) that develops over a short period of time and tends to fluctuate during the course of a day, accompanied by other cognitive impairment such as memory deficit, disorientation, or impairment in language, visuospatial ability, or perception. Disturbance of the sleep-wake cycle (reduced arousal of acute onset or total sleep loss with reversal of the sleep-wake cycle) may also be present.

Ref: FitzGerald JM, Price A. Delirium in the acute hospital setting: the role of psychiatry. BJPsych Advances. 2020 Jul 30;1-11.

Johansson YA et al. Delirium in older hospitalized patients—signs and actions: a retrospective patient record review. BMC geriatrics. 2018 Dec 1;18(1):43.

The correct answer is: Delirium

Question **50**

Not answered

Marked out of 1

An 85-year-old man is admitted to a medical ward with agitation and a recent history of distressing visual hallucinations for the past two days. The most likely diagnosis is

Select one:

- ☐ Delirium tremens
- ☐ Charles Bonnet syndrome
- ☐ Lewy body dementia
- ☐ Vascular dementia
- ☐ Delirium

Your answer is incorrect.

Explanation:

Signs of delirium are disturbed attention, awareness, and cognition that develop over a short period of time and fluctuate in severity. Hyperactive delirium is easiest to recognise with increased psychomotor activity and often mood fluctuations, agitation, refusal to cooperate, disruptive behaviour, disturbance in the sleep-wake cycle, and hallucinations.

Ref: Johansson YA et al. Delirium in older hospitalized patients—signs and actions: a retrospective patient record review. BMC geriatrics. 2018 Dec 1;18(1):43.

The correct answer is: Delirium

Question **51**

Not answered

Marked out of 1

Which of the following scales used in delirium has good symptom coverage and distinguishes delirium from other disorders?

Select one:

- ☐ Delirium Assessment Scale
- ☐ Delirium Rating Scale
- ☐ Delirium Symptom Interview
- ☐ Mini Mental State Examination
- ☐ Confusional State Evaluation

Your answer is incorrect.

Explanation:

DRS (Delirium Rating Scale) and DRS-Revised-98:

- Widely used
- Detects the wide variety of presenting features of delirium, combined with severity scores for each item and global severity scores
- Requires extensive training and psychiatric experience
- Can differentiate delirium from dementia, depression, and schizophrenia

Ref: De J, Wand AP. Delirium screening: a systematic review of delirium screening tools in hospitalized patients. The Gerontologist. 2015 Dec 1;55(6):1079-99.

FitzGerald JM, Price A. Delirium in the acute hospital setting: the role of psychiatry. BJPsych Advances. 2020 Jul 30:1-11.

The correct answer is: Delirium Rating Scale

Question **52**

Not answered

Marked out of 1

In people with Alzheimer disease, the most common psychotic symptoms are

Select one:

- ☐ Delusions
- ☐ Auditory hallucinations
- ☐ Passivity experiences
- ☐ Visual hallucinations
- ☐ Thought alienation phenomenon

Your answer is incorrect.

Explanation:

Delusions, usually of a paranoid nature, occur in approximately 15% of people with Alzheimer disease. Hallucinations (auditory and/or visual) occur in approximately 10-15%.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 157.

The correct answer is: Delusions



## Question 53

Not answered

Marked out of 1

Which of the following syndromes is considered to be an end stage of personality disorder in older people?

Select one:

- ☐ Capgras syndrome
- ☐ DiGeorge's syndrome
- ☐ Diogenes syndrome
- ☐ Charles Bonnet syndrome
- ☐ Van Gogh syndrome

Your answer is incorrect.

Diogenes syndrome: Also called 'senile squalor syndrome' refers to a syndrome of self-neglect in older people, in which eccentric and reclusive individuals become increasingly isolated and neglect themselves, living in filthy, poor conditions. It can be seen as the response of someone with a particular personality type to the hardships of old age and loneliness (Howard and Bergman 1993) and is considered to be an end stage of personality disorder in older people

The correct answer is: Diogenes syndrome





Question **54**

Not answered

Marked out of 1

An 83-year-old woman presents with memory loss, confusion and disorientation. Which of the following features is most suggestive of a delirium?

Select one:

- ☐ Visual hallucinations
- ☐ History of alcohol misuse
- ☐ Disturbance of the sleep-wake cycle
- ☐ Clear consciousness
- ☐ Insidious onset of symptoms

Your answer is incorrect.

Explanation:

Delirium is characterised by disturbed attention (i.e., reduced ability to direct, focus, sustain, and shift attention) and awareness (i.e., reduced orientation to the environment) that develops over a short period of time and tends to fluctuate during the course of a day, accompanied by other cognitive impairment such as memory deficit, disorientation, or impairment in language, visuospatial ability, or perception. Disturbance of the sleep-wake cycle (reduced arousal of acute onset or total sleep loss with reversal of the sleep-wake cycle) may also be present.

Ref: FitzGerald JM, Price A. Delirium in the acute hospital setting: the role of psychiatry. BJPsych Advances. 2020 Jul 30:1-11.

Johansson YA et al. Delirium in older hospitalized patients—signs and actions: a retrospective patient record review. BMC geriatrics. 2018 Dec 1;18(1):43.

The correct answer is: Disturbance of the sleep-wake cycle

## Question 55

Not answered

Marked out of 1

Which of the following is most common in delirium?

Select one:

- ☐ Increased motor activity
- ☐ Labile mood
- ☐ Disturbed sleep-wake cycle
- ☐ Hallucinations
- ☐ Delusions

Your answer is incorrect.

Explanation:

Delirium is characterised by disturbed attention (i.e., reduced ability to direct, focus, sustain, and shift attention) and awareness (i.e., reduced orientation to the environment) that develops over a short period of time and tends to fluctuate during the course of a day, accompanied by other cognitive impairment such as memory deficit, disorientation, or impairment in language, visuospatial ability, or perception. Disturbance of the sleep-wake cycle (reduced arousal of acute onset or total sleep loss with reversal of the sleep-wake cycle) may also be present.

Ref: FitzGerald JM, Price A. Delirium in the acute hospital setting: the role of psychiatry. BJPsych Advances. 2020 Jul 30;1-11.

Johansson YA et al. Delirium in older hospitalized patients—signs and actions: a retrospective patient record review. BMC geriatrics. 2018 Dec 1;18(1):43.

The correct answer is: Disturbed sleep-wake cycle

Question **56**

Not answered

Marked out of 1

Which of the following scale is useful in the assessment of Alzheimer's disease among the learning disabled population?

Select one:

- ☐ CAMCOG
- ☐ DMR
- ☐ NINDS-ADRD criteria
- ☐ AMTS
- ☐ MMSE

Your answer is incorrect.

MMSE has a floor effect and not useful for screening for dementia in Down's syndrome. NINDS criteria are generalised diagnostic criteria for Alzheimer's disease but not a screening tool adapted for Down's syndrome. The Dementia Questionnaire for Mentally Retarded Persons (DMR) is useful for general screening for Alzheimer's disease among the intellectually disabled.

The correct answer is: DMR



Question **57**

Not answered

Marked out of 1

Which of the following is true concerning cholinesterase inhibitors?

Select one:

- ☐ Tacrine is not used due to risk of pulmonary embolism
- ☐ Galantamine does not act directly on nicotine receptors
- ☐ Donepezil is metabolised by CYP2D6 and CYP3A4
- ☐ Ginkgo biloba has the same efficacy as donepezil
- ☐ Memantine is a selective butyrylcholinesterase inhibitor

Your answer is incorrect.

Explanation:

Tacrine is a cholinesterase inhibitor not marketed in the UK due to the potential for hepatotoxicity. RCTs in dementia subjects have shown no benefits from ginkgo biloba on cognition. Ginkgo biloba is not effective in preventing the development of dementia or decline in memory.

| Characteristic of cognitive enhancer       | Donepezil                                                                      | Rivastigmine                                                                           | Galantamine                                                                 | Memantine                                                             |
|--------------------------------------------|--------------------------------------------------------------------------------|----------------------------------------------------------------------------------------|-----------------------------------------------------------------------------|-----------------------------------------------------------------------|
| <b>Primary mechanism</b>                   | AChE-I (selective and reversible)                                              | AChE-I (reversible, non-competitive inhibitor)                                         | AChE-I (competitive and reversible)                                         | NMDA receptor antagonist (non-competitive)                            |
| <b>Other mechanism</b>                     | None                                                                           | BuChE-I                                                                                | Nicotine receptor agonist                                                   | 5-HT <sub>3</sub> receptor antagonist                                 |
| <b>Starting dose</b>                       | 5 mg daily                                                                     | 1.5 mg bd (oral) (or 4.6 mg/24 hours patch)                                            | 4 mg bd solution or 8 mg XL daily                                           | 5 mg daily                                                            |
| <b>Usual treatment dose (and max dose)</b> | 10 mg daily                                                                    | 3-6 mg bd (oral) or 9.5 mg/24 hours patch                                              | 8-12 mg bd solution or 16-24 mg XL daily                                    | 20 mg daily or (10 mg bd)                                             |
| <b>Adverse effects</b>                     | Diarrhoea, nausea, and headache most common                                    | Anorexia, dizziness, nausea, vomiting, and diarrhoea most common                       | Nausea and vomiting most common                                             | Dizziness, headache, fatigue, diarrhoea, and gastric pain most common |
| <b>Half-life (hours)</b>                   | ~70 (permitting once daily dosing)                                             | ~1 (oral) – active for 10 3.4 (patch)                                                  | 7-8 (oral solution) 8-10 (XL capsules)                                      | 60-100                                                                |
| <b>Metabolism</b>                          | CYP3A4 CYP2D6 (minor)                                                          | Minimal involvement of CYP isoenzymes                                                  | CYP3A4 CYP2D6                                                               | Primarily non-hepatic                                                 |
| <b>Drug-drug interactions</b>              | Yes                                                                            | Interactions unlikely                                                                  | Yes                                                                         | Yes                                                                   |
| <b>Effect of food on absorption</b>        | None                                                                           | Delays rate and extent of absorption                                                   | Delays rate but not extent of absorption                                    | None                                                                  |
| <b>Other</b>                               | Absorbed completely from the GI tract<br>Fairly well-tolerated and widely used | More likely to cause GI adverse effects than donepezil due to more peripheral activity | More likely to cause GI and neuropsychiatric adverse effects than donepezil | Generally better tolerated than cholinesterase inhibitors             |

AChE-I = acetylcholinesterase inhibitor  
BuChE-I = butyrylcholinesterase inhibitor

Adapted from Maudsley Prescribing Guidelines, 2021. pp. 605-6.



Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 605-606.

O'Brien et al. Clinical practice with anti-dementia drugs: a revised (third) consensus statement from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2017 Feb;31(2):147-68.

The correct answer is: Donepezil is metabolised by CYP2D6 and CYP3A4

Question **58**

Not answered

Marked out of 1

Between the ages of 65 and 85, the risk of Alzheimer disease

Select one:

- ☐ Quadruples every 5 years
- ☐ Doubles every 10 years
- ☐ Doubles every 5 years
- ☐ Triples every 5 years
- ☐ Triples every 10 years

Your answer is incorrect.

Explanation:

The risk of Alzheimer disease increases with age. The risk is 1% at age 60 years, and then doubles every 5 years. It occurs in 40% of those aged 85 years.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 156.

The correct answer is: Doubles every 5 years

## Question 59

Not answered

Marked out of 1

In progressive supranuclear palsy, which of the following is true?

Select one:

- ☐ Pupils become dilated and fixed
- ☐ A tendency to fall forwards is seen
- ☐ Cervical dystonia occurs
- ☐ Cortical dementia develops
- ☐ Onset is usually in the 4th decade of life

Your answer is incorrect.

Explanation:

Progressive supranuclear palsy (PSP) is a sporadic, progressive, neurodegenerative disease resulting clinically in a symmetrical Parkinson syndrome, prominent postural instability with falls, a vertical gaze palsy, and cognitive decline. Age of onset is usually between 60 and 63. Postural instability and falls are among the cardinal manifestations of PSP. Gait is usually wide-based, and patients tend to fall backwards. Cervical dystonia in PSP is usually retrocollis. The term 'subcortical dementia' was introduced in 1974 to describe the characteristic cognitive PSP profile. This syndrome is different from the ones seen in cortical dementias characteristic of conditions such as Alzheimer or frontotemporal dementias.

Ref: Giagkou N et al. Progressive supranuclear palsy. International review of neurobiology. 2019 Jan 1;149:49-86.

The correct answer is: Cervical dystonia occurs

Question **60**

Not answered

Marked out of 1

Variant CJD is characterised by early

Select one:

- ☐ cognitive symptoms
- ☐ psychotic symptoms
- ☐ behavioural symptoms
- ☐ anxiety and depressive symptoms
- ☐ extra pyramidal symptoms

Your answer is incorrect.

Explanation:

The rise of vCJD followed an epidemic of bovine spongiform encephalopathy (BSE) in cattle. BSE is a prion disease of cows that is thought to have been caused by cattle feeds that contained CNS material from infected cows. The disease in humans affects mainly young people in their 20's and is characterised by early anxiety and depressive symptoms, followed by personality changes, and finally a progressive dementia. Ataxia and myoclonus are prominent, and the typical course is 1-2 yrs until death.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 168.

The correct answer is: anxiety and depressive symptoms



Question **61**

Not answered

Marked out of 1

A 64-year-old patient presents with progressive memory loss, gait disturbance, and urinary incontinence. He has a past history of moderate head injury. Which of the following is correct regarding his current condition?

Select one:

- ☐ Babinski sign is usually present
- ☐ Seizure is a common complication
- ☐ Enlarged ventricles can be seen on neuroimaging
- ☐ Cortical type of dementia is commonly seen
- ☐ Elevated CSF pressure will be noted on lumbar puncture

Your answer is incorrect.

Explanation:

In normal pressure hydrocephalus, there is dilatation of cerebral ventricles (especially third ventricle), but normal CSF pressure at lumbar puncture. The CT scan shows increased size of the lateral ventricles and thinning of the cortex.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 154.

The correct answer is: Enlarged ventricles can be seen on neuroimaging

## Question 62

Not answered

Marked out of 1

A 78-year-old man presents with depression for the first time. His wife says that his depression is very different from the recurrent depression experienced by her 44-year-old son. Which one of the following is a feature seen commonly in early onset depression when compared with late-onset depression?

Select one:

- ☐ Frontal lobe dysfunction
- ☐ Executive dysfunction
- ☐ Episodic memory dysfunction
- ☐ None of the above
- ☐ Attentional problems

Your answer is incorrect.

Although depression in all ages is associated with some degree of impaired concentration and subjective difficulties with memory, these cognitive deficits seem to occur when first onset is in older age (Holroyd and Duryee 1997). Other cognitive deficits such as episodic memory disturbance and executive dysfunction are also commonly found in depression. Studies also found that individuals with late-onset depression had specific deficits in attention and executive functions (Consistent with frontal lobe dysfunction) whereas those with recurrent early-onset depression exhibited deficits in episodic memory (Rapp et al 2005) which is consistent with temporal lobe dysfunction.

The correct answer is: Episodic memory dysfunction

Question **63**

Not answered

Marked out of 1

A 75-year-old man with a history of vascular dementia complains of drooping of the corner of the mouth and difficulty closing his left eye. He is able to frown. Which cranial nerve palsy is more likely to present in this way?

Select one:

- ☐ Facial nerve palsy (upper motor neurone)
- ☐ Facial nerve palsy (lower motor neurone)
- ☐ Trigeminal nerve palsy (sensory)
- ☐ Oculomotor nerve palsy
- ☐ Trigeminal nerve palsy (motor)

Your answer is incorrect.

In facial nerve palsy with a UMN lesion, the lower part of the face on the contralateral side will be affected, but the upper part spared due to the bilateral control of the upper facial muscles (frontalis and orbicularis oculi). Patients with weak smile but that are able to close their eyes tightly and wrinkle their forehead symmetrically are said to have "central facial weakness," as they likely have UMN pathway damage.

LMN lesion can result in a CN-VII palsy manifested as both upper and lower facial weakness on the same side of the lesion

The correct answer is: Facial nerve palsy (lower motor neurone)

Question **64**

Not answered

Marked out of 1

Which factor is most likely to increase the risk of very-late-onset schizophrenia-like psychosis?

Select one:

- ☐ Low socioeconomic status
- ☐ Substance misuse
- ☐ Poor pre-morbid functioning
- ☐ Female sex
- ☐ Social isolation

Your answer is incorrect.

Explanation:

Female sex, increasing age, sensory impairment, and migrant status are risk factors for very-late-onset schizophrenia-like psychosis (VLOS). In contrast to schizophrenia, genetic factors have not been implicated in the aetiology of VLOS.

Ref: Howard R, et al. Antipsychotic treatment of very late-onset schizophrenia-like psychosis (ATLAS): a randomised, controlled, double-blind trial. The Lancet Psychiatry. 2018 Jul 1;5(7):553-63.

The correct answer is: Female sex

## Question 65

Not answered

Marked out of 1

Which of the following clinical features is not consistent with an ICD-11 diagnosis of delirium?

Select one:

- ☐ Behavioural symptoms
- ☐ Focal neurocognitive impairment
- ☐ Disturbance of the sleep-wake cycle
- ☐ Disturbance of emotion
- ☐ Impairment of attention and awareness

Your answer is incorrect.

Explanation:

Delirium is characterised by a disturbance of attention, orientation, and awareness that develops within a short period of time (e.g., within hours or days), typically presenting as significant confusion or global neurocognitive impairment, with transient symptoms that may fluctuate depending on the underlying causal condition or aetiology. In delirium, cognition is typically impaired in a global manner, such that multiple areas of neurocognitive functioning are impaired upon assessment. A disturbance of the sleep-wake cycle, including reduced arousal of acute onset or total sleep loss with reversal of the sleep-wake cycle, may also be present. Delirium often includes disturbance of emotion, including anxiety symptoms, depressed mood, irritability, fear, anger, euphoria, or apathy. Behavioural symptoms may be present (e.g., agitation, restlessness, impulsivity).

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: Focal neurocognitive impairment

## Question 66

Not answered

Marked out of 1

Which of the following tests would be required to make a diagnosis of Huntington disease in a patient without a family history?

Select one:

- ☐ Immunological testing
- ☐ Electromyography
- ☐ EEG
- ☐ Gene testing
- ☐ Sleep studies

Your answer is incorrect.

Explanation:

Diagnosis of Huntington disease (HD) is straightforward if a patient with a known family history of HD presents with the typical neurological movement disorder. Gene testing is not required if there is laboratory confirmation of the Huntington disease gene in the family. In a patient without a family history, gene testing is essential but does not give a clinical diagnosis and must be combined with clinical history and physical exam.

Ref: Bachoud-Lévi AC et al. International Guidelines for the treatment of Huntington's Disease. Frontiers in neurology. 2019;10:710.

The correct answer is: Gene testing

Question **67**

Not answered

Marked out of 1

A 69-year-old woman lost her husband recently. Which of the following clinical features would support a diagnosis of depression rather than a normal grief reaction in this patient?

Select one:

- ☐ Diminished appetite
- ☐ Insomnia
- ☐ Auditory hallucinations of her dead husband's voice
- ☐ Anhedonia
- ☐ Intense guilt

Your answer is incorrect.

Explanation:

Grief can present with an overlap of symptoms characteristic of a major depressive episode. Typical symptoms associated with grief include feelings of sadness, insomnia, diminished appetite, and, in some cases, weight loss. Individuals may become withdrawn and avoid even their favourite activities. Symptoms indicating major depressive disorder exceeding typical grief include intense guilt related to issues beyond those surrounding the death of the loved one, preoccupation with death other than thoughts about being dead to be with the deceased person, morbid preoccupation with worthlessness, marked psychomotor retardation, prolonged severe functional impairment, and hallucinations other than transient perceptions of the voice of the deceased person.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 176.

The correct answer is: Intense guilt

Question **68**

Not answered

Marked out of 1

Which of the following clinical features is common to both delirium and dementia?

Select one:

- ☐ Rapid onset
- ☐ Global cognitive impairment
- ☐ Clear consciousness
- ☐ Clouding of consciousness
- ☐ Gradually progressive impairment over a period of months

Your answer is incorrect.

Explanation:

Both dementia and delirium are characterized by global cognitive impairment.





| Feature                                             | Delirium                                                                                                                                                                                                                                     | Dementia                                                                                                                                                                                                                               |
|-----------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Onset</b>                                        | Abrupt, though initial loss of mental clarity may be subtle                                                                                                                                                                                  | Insidious and progressive                                                                                                                                                                                                              |
| <b>Duration</b>                                     | Hours to days (though can be prolonged)                                                                                                                                                                                                      | Months to years                                                                                                                                                                                                                        |
| <b>Attention</b>                                    | Reduced ability to focus, sustain, or shift attention is a hallmark feature, occurring early in presentation                                                                                                                                 | Normal unless severe dementia                                                                                                                                                                                                          |
| <b>Consciousness (awareness of the environment)</b> | Fluctuating (making assessment at multiple timepoints necessary), reduced level of consciousness, and impaired orientation                                                                                                                   | Generally intact                                                                                                                                                                                                                       |
| <b>Speech</b>                                       | Incoherent, disorganized<br>Distractible in conversation                                                                                                                                                                                     | Ordered, may develop anomia or aphasia                                                                                                                                                                                                 |
| <b>Other features</b>                               | Caused by underlying medical condition, substance, or medication side effect<br>Hyperactive, hypoactive, and mixed forms of psychomotor disturbance are possible<br>Disruption in sleep duration and architecture<br>Perceptual disturbances | Caused by underlying neurological process (e.g. beta-amyloid plaque accumulation in Alzheimer disease), with symptoms varying depending on underlying pathologies (e.g. fluctuations in cognition are a feature of Lewy body dementia) |

Adapted from Fong et al. 2015



Ref: Fong TG et al. The interface of delirium and dementia in older persons. The Lancet. Neurology. 2015 Aug;14(8):823.

The correct answer is: Global cognitive impairment

## Question 69

Not answered

Marked out of 1

Which of the following can be used as the first choice medication for elderly patients with delirium in a surgical ward?

Select one:

- ☐ Haloperidol
- ☐ Olanzapine
- ☐ Chlorpromazine
- ☐ Lorazepam
- ☐ Risperidone

Your answer is incorrect.

Explanation:

Psychotropic medication should be used with caution in delirium and only for situations that have not yielded to non-pharmacological interventions. Conventionally, the antipsychotic of choice is haloperidol, given its absence of anticholinergic side effects. The use of antipsychotics is controversial, and there is little evidence to indicate that they should be used to treat delirium directly. There is no evidence to support the use of benzodiazepines in managing delirium not associated with alcohol or benzodiazepine withdrawal.

Ref: FitzGerald JM, Price A. Delirium in the acute hospital setting: the role of psychiatry. BJPsych Advances. 2020 Jul 30:1-11.

The correct answer is: Haloperidol

## Question 70

Not answered

Marked out of 1

The similarities between early onset and late onset schizophrenia includes all of the following except

Select one:

- ☐ Responding to antipsychotics
- ☐ Associated personality traits
- ☐ Predominance of positive symptoms
- ☐ Subtle neuroimaging findings
- ☐ Having similar sex distribution

Your answer is incorrect.

Late-onset schizophrenia is characterised by (Palmer et al. 2001) fewer negative symptoms, better response to antipsychotics and better neuropsychological performance. But it is similar to the usual onset disease in terms of genetic risk, presence and severity of positive symptoms, early psychosocial maladjustments and subtle brain abnormalities revealed by imaging.

The correct answer is: Having similar sex distribution



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